CLEAR FORM

aetna

CT/CTA/MRI/MRA PRIOR AUTHORIZATION FORM

SECTION 1. MEMBER DEMOGRAPHICS

Patient Name (First, Last):		DOB: Sample 10
Health Plan:	Member ID:	Group #:

SECTION 2. ORDERING PROVIDER INFORMATION

Physician Name (First, Last):		
Primary Specialty:	NPI:	Tax ID:
Phone #:	Fax #:	Contact Name: Sample 3

SECTION 3. FACILITY INFORMATION

Facility Name:	Facility Tax ID:	NPI:	
Address: Sample 1	City: Sample 2	State:	Zip:
Phone #:	Fax #:	Date of Service: Sample 7	

SECTION 4. EXAM REQUEST

☒ CT ☒ MRI ☒ CTA ☒ MRA

CPT Code(s): Sample 4

Description: Sample 8

ICD Diagnosis Code(s):

Description: Sample 9

Date of first office visit for this condition with any provider: Sample 5

Date of most recent office visit for this condition with any provider: Sample 6

SECTION 5. SELECT APPLICABLE BODY REGION AND CHECK REASON(S) FOR STUDY *(CHECK ALL THAT APPLY)*

☒ ABDOMINAL/ PELVIS

Abd/Pelvis Combination Study ☒ Yes ☐ No

- ☒ Acute Pain (less than 48 hrs)
- ☒ Hematuria
- ☒ Inflammatory Bowel Disease consistent with Appendicitis, Diverticulitis, or Abscess
- ☒ Suspected Hemochromatosis
- ☒ Abdominal or Pelvic Mass
- ☒ Suspected Vascular Disease, Mesenteric Ischemia
- ☒ Suspected Renal Artery Stenosis
- ☒ Hernia
- ☒ Pancreatic or adrenal mass seen on other imaging
- ☒ Chronic Pain (more than 48 hours)
- ☒ Abdominal/Pelvic Trauma
- ☒ Anemia
- ☒ Fever of Unknown Origin [FUO]
- ☒ Ascites
- ☒ Prostate Neoplasm
- ☒ Pre- or post-OP evaluation
- ☒ Lower extremity edema
- ☒ Significant weight loss (10% of body weight over 6 months or less)
- ☒ Transplant
- ☒ Kidney/Urethral Obstruction or Calculus
- ☒ Jaundice, Abnormal Liver Function Tests
- ☒ Endometrial Abnormality
- ☒ Staging (malignancy)
- ☒ Suspected Aneurysm/Dissection/AVM
- ☒ MRCP
- ☒ Lower extremity claudication
- ☒ Suspected abnormality of pelvic bones or muscular structures
- ☒ Pelvic Floor Dysfunction
- ☒ Other (describe): _____

☒ SPINE

- ☒ Neurological Deficits
- ☒ Known or suspected infection
- ☒ Persistent Pain
- ☒ Radiculopathy
- ☒ Possible Fracture
- ☒ Other (describe): _____
- ☒ Trauma or recent injury
- ☒ Known or suspected tumor on bone scan or x-ray
- ☒ Unilateral Muscle wasting
- ☒ Pre- or post-OP Evaluation
- ☒ Suspected Multiple Sclerosis (not applicable for CT or for CT or MRI of lumbar region)

PRIOR /CURRENT TREATMENT(S)

Check One (Prior Treatment)

- ☒ No Prior Treatment
- ☐ 3–5 weeks of treatment
- ☐ 6 or more weeks of treatment

Check all treatments that apply

- ☒ NSAIDS
- ☒ Spine Injections
- ☒ Home Exercise Program
- ☒ Physical Therapy
- ☒ Chiropractic Treatment
- ☒ Oral Steroid

☒ BREAST MRI DIAGNOSTIC ☒ BREAST MRI SCREENING

- ☒ Abnormal/inconclusive mammogram or ultrasound
- ☒ Suspected Recurrence of Breast Cancer
- ☒ Mass evaluation post surgery
- ☒ Evaluate extent of invasive cancer
- ☒ Evaluation axillary node metastasis
- ☒ Dense breast tissue
- ☒ Evaluation of symptomatic patients with breast implants, for detection of implant rupture
- ☒ Positive Margins Post-OP
- ☒ 6 months follow up abnormal MRI (birads3)

(continued on next page)

☒ REQUEST FOR ANNUAL SCREENING FOR BREAST CANCER *(If yes, check reason(s) below)*

- ☒ Lifetime risk 20% or greater as defined by BRACA PRO or other models
- ☒ *BRCA1* and *BRCA2* mutation
- ☒ History of lobular or ductal carcinoma in situ on biopsy
- ☒ Li-Fraumeni Syndrome, Cowden Syndrome
- ☒ Radiation therapy to chest between ages 10–30
- ☒ Bannayan-Riley-Ruvucaba Syndrome

☒ BRAIN/HEAD

- ☒ Known or suspected tumor/mass or metastasis
- ☒ Recent significant head trauma
- ☒ Known or suspected stroke
- ☒ Brain infection or abscess
- ☒ Abnormal neurological exam
- ☒ **New** onset of seizures
- ☒ Pre- or post-OP evaluation
- ☒ Suspected Multiple Sclerosis (not for CT)
- ☒ Follow up treatment (surgery/chemotherapy/radiation)
- ☒ Breakthrough seizures
- ☒ Vascular abnormalities (AVM Aneurysm Dissection Stenosis, Obstruction)
- ☒ Suspected acoustic neuroma
- ☒ Suspected pituitary adenoma and elevated prolactin (>20 ng/ml)

New Headache: ☒ With fever ☒ With exertion ☒ On awakening ☒ Focal neurological findings ☒ Worst headache of life (thunderclap)

Chronic Headache: ☒ New neurological findings ☒ New syncope ☒ New mental status changes

☒ CHEST

- ☒ Chest wall or pleural mass
- ☒ Follow up trauma
- ☒ Significant Hemoptysis
- ☒ Persistent unexplained wheeze
- ☒ Lesion on chest x-ray suggestive of malignancy or metastatic disease
- ☒ Standard staging or post therapy follow-up for patient with a pathologically proven malignancy
- ☒ Congenital Heart Disease
- ☒ Acquired Pediatric Heart Disease
- ☒ Suspected vascular abnormality, aneurysm, AVM, congenital anomaly
- ☒ Suspected Pulmonary Embolus
- ☒ Persistent infiltrate/pneumonia despite 4–6 weeks antibiotic therapy
- ☒ Suspected/known asbestostis or other pneumoconiosis

Chest x-ray results:

- ☒ Normal ☒ Abnormal
- ☒ Not performed in past 2 months
- ☒ Pre- or post-OP evaluation
- ☒ Mediastinal mass
- ☒ Screening for lung nodules
- ☒ Lung abscess or inflammatory process
- ☒ Chest x-ray or PFT suggestive of pulmonary fibrosis
- ☒ Signs or symptom suggestive of lung cancer (unintentional weight loss, anemia, paraneoplastic syndrome, etc.)
- ☒ Other (describe): _____

☒ SINUS, FACE, NECK, ORBIT

- ☒ Follow up — Trauma
- ☒ Painful swallowing
- ☒ Staging of malignancy
- ☒ Known or suspected tumor (Palpable Neck Mass)
- ☒ Vascular abnormalities (AVM Aneurysm Dissection Stenosis, Obstruction)
- ☒ Pre- or post-OP evaluation
- ☒ Salivary gland mass or stone
- ☒ Suspected thyroid mass
- ☒ Possible infection or abscess
- ☒ Immunocompromised patient or fungal infection warranting MR

☒ **Sinusitis**

- ☒ Acute (less than 3 months)
- ☒ Chronic (more than 3 months)
- ☒ Recurrent — (4 or more episodes/yr)

☒ **Sinusitis Treatment**

- ☒ No antibiotic treatment
- ☒ Failure single course antibiotics
- ☒ Failure 2 or more courses antibiotics

☒ Other (describe): _____

☒ UPPER/ LOWER EXTREMITIES

- ☒ Recent trauma
- ☒ Palpable soft tissue mass
- ☒ Joint locking
- ☒ Joint infection/inflammation
- ☒ Avascular/Aseptic Necrosis
- ☒ Charcot joint
- ☒ Ligament, tendon, or fibrocartilage tear
- ☒ Pre- or post-OP evaluation
- ☒ Soft tissue abscess
- ☒ Tarsal coalition (feet)
- ☒ Requested as part of arthrogram
- ☒ Meniscal or labral tear
- ☒ Abnormal plain film, bone scan, or ultrasound
- ☒ Rotator cuff tear (shoulder)
- ☒ Known or suspected tumor, metastasis
- ☒ Fracture evaluation
- ☒ Suspected vascular abnormality (aneurysm dissection, thromboembolic disease, A-V malformation or fistula vasculitis, ischemia, pre or post op, venous thrombosis)
- ☒ Other (describe): _____

Upper/Lower Extremities X-Ray Results: ☒ Normal ☒ Abnormal ☒ Not performed ☒ Not performed in the past 2 months

☒ PERSISTENT PAIN AND/OR DISABILITY *(IF YES, CHECK REASON(S) BELOW)*

Prior Treatment ***(Check One)***

- ☒ No prior treatment
- ☐ 3–5 weeks of treatment
- ☐ 6 or more weeks of treatment

Check all treatments that apply.

- ☒ NSAIDS
- ☒ Splinting/brace/sling
- ☒ Home exercise program
- ☒ Physical therapy
- ☒ Chiropractic treatment
- ☒ Oral/Intra-articular Steroids

SECTION 6. DOCUMENT EXAM FINDINGS, PRIOR TESTS, RESULTS, AND DATES *(INCLUDE TREATMENT DESCRIPTION FOR CONSERVATIVE THERAPY DURATION, PRIOR IMAGING, AND ANY TRAUMA HISTORY)*

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_____

_____

Providers should consult the health plan's coverage policies, member benefits, and medical necessity guidelines to complete this form. Providers may attach any additional data relevant to medical necessity criteria.